

REPUBLIC OF KENYA
CHIEF MAGISTRATE COURT AT NAIROBI
MILIMANI COMMERCIAL COURT
CIVIL SUIT NO. E5246 OF 2022

EBBY NAKHUMWA MAKOKHA-----PLAINTIFF

VERSUS

NAIROBI WOMEN'S HOSPITAL-----DEFENDANT

JUDGEMENT

The plaintiff initiated legal proceedings against the defendant through a plaint dated October 19, 2023. The plaintiff sought general damages, aggravated damages, and exemplary damages. In response, the defendant filed a statement of defense dated November 10, 2022, refuting all the claims made by the plaintiff. The defense requested the court to dismiss the plaintiff's case and impose costs. Subsequently, a full trial ensued, during which both the plaintiff and the defendant presented their witnesses on various dates.

On Evidence

Plaintiff's case

The plaintiff testified that on June 11, 2017, she visited the defendant's Adams Arcade branch for prenatal care, accompanied by her husband and son. During this visit, it was determined that the unborn child had a regular heartbeat and weighed 4.05 kg. However, due to a delay in delivery at 40 weeks of gestation, the doctor referred her to the main branch at Hurlingham for induction. An ambulance transported her to the Hurlingham branch, where she was induced around 8:30 pm by the doctor's nurse under Dr. Igogo's supervision. Despite expressing discomfort and severe pain to the attending nurse, her concerns were disregarded. The following day, she underwent surgery and in the process she lost her baby and suffered a ruptured uterus.

Subsequently, the plaintiff filed a case against Dr. Igogo with the Kenya Medical Practitioners and Dentist Council. The ruling, delivered on February 21, 2022, exonerated Dr. Igogo but held the defendant's staff responsible for her loss and

damages. The defendants were fined and instructed to prepare for the plaintiff's compensation, which they had neglected or refused to pay. As evidence, the plaintiff submitted an order granting her leave to file a suit out of time, an application to file a complaint, the ruling dated 21st February 2022, and a demand letter.

Defence case

Dr Peter Igogo, a resident obstetrician/gynaecologist at the defendant's Hurlingham branch, provided testimony that the plaintiff was admitted to the defendant's facility on June 11, 2017. Dr. Igogo examined her on June 12, 2017. According to his account, the patient had been admitted with a urinary tract infection during pregnancy at 40 weeks and 5 days. She was induced using PGE2. Dr Igogo assessed her at 9:15 am and was subsequently called by the midwife at 12:30 pm due to poor progress in labor. The patient experienced three strong contractions within a 10-minute interval, and the descent of the foetus remained at 3/5. Additionally, the fetal heart rate was irregular at 89 beats per minute. Dr Igogo diagnosed poor labor progress and prepared the patient for an emergency Caesarean section. The exhaustion plan, along with potential complications, was explained, and informed consent was obtained. The patient underwent surgery at 12:50 pm and 12:58 pm. While in the post-anaesthesia care unit (PACU), she suffered two generalized tonic-clonic convulsions and elevated blood pressure, leading to a diagnosis of eclampsia. After consultation with the anaesthetist, blood samples were taken for testing, and the patient was stabilized. An emergency C-section was performed under general anaesthesia. During the surgery, they discovered a ruptured uterus extending from the fundus to the cervical region, with both the male fetus and placenta extruded into the abdominal cavity. Unfortunately, the outcome was a fresh stillbirth. Dr Maugo, a consultant paediatrician, attempted resuscitation but was unsuccessful. Due to the severe uterine rupture and active bleeding, repairing the uterus was not feasible, and the decision was made to remove it to save the mother's life.

"The patient received admission to the High Dependency Unit (HDU) for close monitoring. A family conference took place on the same day, during which the husband and the patient's mother were informed of the outcome. They strongly

advised the hospital not to disclose the news to the patient, fearing it might impact her recovery. However, on the second post-operation day, after her stabilization, the patient was informed of the situation. She received counselling and was eventually discharged on the fifth post-operation day. A follow-up review after two weeks confirmed her well-being.

According to Dr Peter Igogo, the patient's management adhered to guidelines from the Ministry of Health (MOH), the World Health Organization (WHO), and other relevant authorities. As evidence, the witness presented an ultrasound report, the ob-gyn's medical report, and the patient's discharge summary sheet in court.

During cross-examination, Dr Igogo clarified that the case filed with the Kenya Medical Practitioners and Dentists Council exonerated him personally but attributed negligence to the hospital. He acknowledged that the defense had not submitted records of the patient's urinary tract infection (UTI) results. Additionally, he admitted informing the board that the administration of Oxytocin in the context of obstructed labor might have contributed to the uterine rupture.

Issues for determination

1. Whether the defendants and their agents were negligent
2. Whether the plaintiff is entitled to the damages sought in the plaint.
3. Who should bear the costs of the suit?

Determination

Whether the defendants and their agents were negligent

In her evidentiary testimony, the plaintiff described her visitation to the hospital operated by the defendant for childbirth. Regrettably, during the delivery, she suffered the dual misfortune of losing both her infant and her uterus, attributing these lamentable outcomes to the alleged negligence on the part of the defendant. Furthermore, in the second paragraph of her plaint, the plaintiff expounded upon the specific instances of professional negligence, setting forth the particulars in a subsequent manner:

1. Failed in their duty of care towards the plaintiff

2. Inappropriate augmentation of labour-provoking uterine rupture resulting in a fresh stillbirth
3. Administering oxytocin without the attending doctor's prescription
4. Caused the plaintiff to undergo emergency caesarian
5. Failed to respond to the plaintiff's complaints in good time leading to the rupture of her uterus
6. Causing the plaintiff to suffer eclampsia
7. Caused the loss of the plaintiff's unborn child who was delivered as a fresh stillbirth

The plaintiff contends that the purported departures by the defendant from the recognized standard of care serve as the immediate and direct causation of the profound and irremediable injuries sustained throughout the childbirth procedure. The plaintiff articulates that she endured the tragic loss of her unborn child, irreversible uterine rupture, emotional distress, and an incapacity to conceive.

In substantiation of her assertions, the plaintiff has produced a ruling from the Kenya Medical Practitioners and Dentist Council, which, in its findings, held the defendant as having exhibited negligence in the pertinent matter. The plaintiff implores the court to find the defendant culpable for professional negligence, thereby attributing liability for the resultant loss and damages suffered by the plaintiff.

The defence through Dr. Peter Igogo denied being negligent in handling the plaintiff. He stated that the plaintiff was managed according to MOH, WHO, ALSO and other guidelines. The defendant in submissions stated that the ruling by the Kenya Medical Practitioners and Dentist Council was not formally proven hence the same amounts to hearsay. The defendant cited the following cases in support of its submissions:

1. Abdirahman N Mohammed & Another vs Antonina Akinyi Oruko(2019) eKLR
2. The Estate of Charles Ndegwa Kiragu alias Ndegwa Kiragu deceased (2016) eKLR which case dealt with secondary evidence.

Having duly examined the evidence presented and the opposing contentions tendered, the court ascertains that the defendant acknowledges having provided

medical treatment to the plaintiff at its establishment on the 12th of June 2017. Furthermore, the admission is made that the plaintiff underwent induction and subsequently underwent an emergency cesarean section following an extended period of labor. Additionally conceded is the unfortunate outcome wherein the plaintiff experienced the loss of her unborn child and underwent the removal of her uterus.

In Kenneth Nyaga Mwise v Austin Kiguta & 2 others [2015] eKLR the Court of Appeal expressed itself as follows;

"The mere marking of a document for identification does not dispense with the formal proof thereof. How does a document become part of the evidence for the case? Any document filed and/or marked for identification by either party, passes through three stages before it is held proved or disproved. First, when the document is filed, the document though on file does not become part of the judicial record. Second, when the documents are tendered or produced in evidence as an exhibit by either party and the court admits the documents in evidence, it becomes part of the judicial record of the case and constitutes evidence; mere admission of a document in evidence does not amount to its proof; admission of a document in evidence as an exhibit should not be confused with proof of the document. Third, the document becomes proved, not proved or disproved when the court applies its judicial mind to determine the relevance and veracity of the contents - this is at the final hearing of the case. When the court is called upon to examine the admissibility of a document, it concentrates only on the document. When called upon to form a judicial opinion whether a document has been proved or disproved or not proved, the Court would look not at the document alone but it would take into consideration all facts and evidence on record."

In this case, the plaintiff produced the ruling dated 21st February 2022. The court notes that the defendant did not at any point object to the production of the said document. The only issue is for the court to determine if the plaintiff provided evidence in support of the ruling.

Section 20 of the Medical Practitioners and Dentists Act, Cap 253 which states that:

(1) Any person who is dissatisfied with any professional service offered, or alleges a breach of standards by a registered or licensed person under this Act, may lodge a complaint in the prescribed manner to the Council.

(2) The Council may, or through a committee appointed for that purpose, inquire into any complaint of professional misconduct, malpractice or any breach of standards.

(3) Upon an inquiry held by the Council to determine the complaint made under subsection (2), the person whose conduct is being inquired into shall be afforded an opportunity of being heard, either in person or through a representative.

(4) For purposes of proceedings at any inquiry held under this section, the Council may administer oaths, enforce the attendance of witnesses and production of books and documents.

(5) The Council shall regulate its own procedure in disciplinary proceedings.

Further, Rule 4A (1) of the Medical Practitioners and Dentists (Disciplinary Proceedings) (Procedure) Rules, 1979 establishes the Professional Conduct Committee whose functions are to:

(a) conduct inquiries into the complaints within such counties as the Board may specify and make appropriate recommendations to the Board;

(b) ensure that the necessary administrative and evidential arrangements have been met so as to facilitate the Board to effectively undertake an inquiry under rule 6;

(c) convene sittings in respective counties to determine complaints;

(d) promote arbitration between the parties and refer matters to such arbitrator as the parties may in writing agree.

The court has verified, based on the testimony of both the plaintiff and the defense witness, that the proceedings in question occurred, and the board rendered a decision regarding the matter under consideration. It is also admitted by the defence that they have not appealed against the said decision. Considering the context of this case, I conclude that the board's ruling has been substantiated.

On whether the defendant owed a duty of care to the plaintiff

On whether the defendants owed the plaintiff a duty of care in the case of *Ricarda Njoki Wahome (suing as the administrator of the estate of the late Wahome Mutahi(deceased) vs. Attorney General & 2 others (2015) eKLR*, the court had this to say;

"A duty of care arises once a doctor or other health care professional agrees to diagnose or treat a patient. That professional assumes a duty of care towards that patient."

With respect to a duty of care owed by a medical practitioner to a patient, *Halsbury's Law of England, Vol. 26 at page 17* states thus;

"A person who holds himself as ready to give medical advice or treatment impliedly undertakes that he is possessed of skill and knowledge for that purpose. Such a person, whether he is a registered medical practitioner or not, who is consulted by a patient, owes him certain duties namely, a duty of care in deciding whether to undertake the case, a duty of care in deciding what treatment and a duty of care in his administration of that treatment."

Further, in the case of *Jimmy Paul Semanya vs. Aga Khan Hospital & 2 others (2006)eKLR* the court held;

"There exist a duty of care between the patient and the doctor, hospital or health provider once this relationship has been established, the doctor is taken to;

- a) Possess the medical knowledge required of a reasonably competent medical practitioner engaged in the same speciality.*
- b) Possess the skills required of a reasonable competent health care practitioner engaged in the same speciality.*
- c) Exercises the care in the application of the knowledge and skill to be expected of a reasonably competent health care practitioner in the same speciality and;*

- d) Use the medical judgment in the exercise of that care required of a reasonably competent practitioner in the same medical or health care speciality.

To define a duty of care in medical negligence;

"A physician has a duty of care and skill which is expected reasonably of a competent practitioner in the same class to which a physician belongs acting in the same or similar circumstances. When a physician or other medical staff member does not treat a patient with proper amount of quality care, resulting in serious injury or death, they commit medical negligence."

The duty of care expected of a medical person was also explained in the case of R. vs. Bateman (1925) 94 EJ KB791 as follows;

"if a person holds himself out as possessing special skill and knowledge and he is consulted, as possessing such skills and knowledge, by or on behalf of a patient, he owes a duty to that patient to use due caution in undertaking the treatment. If he accepts the responsibility and undertakes the treatment and the patient submits to his direction and treatment accordingly, he owes a duty of care to the patient to use diligence, care, knowledge, skills and caution in administering the treatment."

Guided by the above authorities, this court finds that the defendant owed the plaintiff a duty of care.

In **Blyth v Birmingham Co. [1856] 11 exch. 781- 784**. Negligence was defined as the act of doing something or an omission by a reasonable man, guided upon considerations which regulate the conduct of human affairs. Further, that in a case of negligence there should be a duty of care owed, a breach of that duty and damage suffered by the person to whom the duty was owed.

In **Pope John Paul's Hospital & another v Baby Kasozi [1974] EA 221**, as cited by **G.V. Odunga, J in Herman Nyangala Tsuma (supra)**, the East African Court of Appeal held that:

"If a professional man professes an art, he must reasonably be skilled in it. He must also be careful, but the standard of care, which the law requires, is not insurance against accidental slips. It is such a degree of care as normally skilful member of the profession may reasonably be expected to exercise in the actual circumstances of the case, and, in applying the duty of care to the case of a surgeon, it is peculiarly necessary to have regard to the different kinds of circumstances that may present themselves for urgent attention...A charge of professional negligence against a medical man was serious. It stood on a different footing to a charge of negligence against the driver of a motor car. The consequences were far more serious. It affected his professional status and reputation...The burden is to prove that the damage was caused by negligence and was not a question of misadventure, and that burden must be discharged on a preponderance of evidence...In medical cases the fact that something has gone wrong is not in itself any evidence of negligence."

In Bolam v Friern Hospital Management Committee [1957] 1 WLR 582 McNair, J stated as follows;

"test of whether there has been negligence or not is not the test of the man on top of the Clapham omnibus, because he has not got this special skill. The test is the standard of the ordinary skilled man exercising and professing to have that special skill."

The duty of care expected of a medical person was also explained in the case of **R. vs. Bateman (1925) 94 EJ KB791** as follows;

"if a person holds himself out as possessing special skill and knowledge and he is consulted, as possessing such skills and knowledge, by or on behalf of a patient, he owes a duty to that patient to use due caution in undertaking the treatment. If he accepts the responsibility and undertakes the treatment and the patient submits to his direction and treatment accordingly, he owes a duty of care to the patient to use diligence, care, knowledge, skills and caution in administering the treatment"

The plaintiff claimed that she went to the hospital for delivery, expecting the defendant to provide competent medical care for a safe birth. However, the plaintiff

alleged that this did not happen as expected. The plaintiff left the hospital without her baby and uterus, blaming the tragic outcome on what she considered professional negligence by the defendant's employees. According to the plaintiff's submitted ruling, the committee determined that a nurse gave oxytocin to the patient without the doctor's prescription. The committee found such a prescription to be unsuitable and in breach of expected standards of care. They also stated that the administration of oxytocin infusion in a patient with obstructed labor caused a uterine rupture, and the affected fetal head may have blocked the proper flow of blood through the vaginal canal. They censured the defendant for improper augmentation of labour in the patient which obstructed labour which induced the uterus rupture with expulsion of the placenta and resulted in a fresh stillbirth. Based on the evidence presented, this court concludes that the plaintiff has established on a balance of probability that the defendant was negligent as per the particulars stated in the plaint. The court finds the defendant liable. The court is guided by the following authority in holding the defendant liable for the actions of its employees;

Ricada Njoki Wahome vs. Attorney General & 2 others (Supra) which cited the case of **M (a minor) vs. Amulega & Another (2001) eKLR 420** where the court held;

"Authorities who own a hospital are in law under a self-same duty as the humblest doctor. Whenever they accept a patient for treatment, they must use reasonable care and skill to cure him of his ailment. The hospital authorities cannot of course do it by themselves. They must do it by the staff whom they employ and if their staff is negligent in giving treatment they are just as liable for that negligence as is anyone else who employs others to do his duties for him..... it is established that those conducting a hospital are under a direct duty of care to those admitted as patients to the hospital. They are liable for the negligent acts of a member of the hospital staff, which constitutes a breach of duty of care owed by him to the plaintiff thus there has been acceptance from the courts that hospital authorities are in fact liable for breach of duty by its members of staff. It is trite law that a medical practitioner

owes a duty of care to his patients to take all due care, caution and diligence in the treatment "

Whether the plaintiff is entitled to the damages sought in the plaint

General damages

The plaintiff in this case stated that she suffered the loss of her unborn child, irreversible rupture of the uterus, emotional and mental stress and the inability to conceive. She submitted that due to the defendant's negligence, she suffered loss and damage. The plaintiff in her submission urged the court to award general damages of Kshs.3,500,000/= The plaintiff cited the following cases;

PKM (Suing on own behalf and as next friend of AJB) & GSM vs Nairobi Women Hospital & Mutinda (2018) eKLR where the court awarded Kshs.8,000,000/= for medical negligence.

Hellen Kirimana vs PCEA Kikuyu Hospital (2016) eKLR where a sum of Kshs.2,000,000/= was awarded for pain and suffering

BS vs Jonardan D Patel (2019) eKLR where Kshs.2,000,000/= was awarded for pain and suffering.

The defence on their part urged the court to dismiss the claim. It was further submitted that the claimant should be bound by the pleadings since she had only pleaded for damages for losing her child and for the loss of her uterus. On their part, the defendants urged the court to award general damages of Kshs.1,500,000/= The defendant relied on the following authorities;

Beatrice Wachira & another vs Attorney General & Another (2020) eKLR where the court reviewed an award granted to the plaintiff from Kshs.500,000/= to Kshs.1,500,000/=

I have considered the rival submissions. This court is guided by the decision in **Osman Mohammed & Another vs. Saluto Bandit Mohamud, ca. 30/97** where the following passage, in the case of **Kigaragari vs. Aya (1982 – 1988) KAR 768** is employed

"Damages must be within limits set out by decided cases and also within the limits the Kenyan economy can afford. Large awards are inevitably passed on the members of the public, the vast majority of whom cannot afford the burdens in the form of increased costs of insurance or increased fees."

Courts have often held that

"Damages should not be so inordinately low or so inordinately high as to be a wholly erroneous estimation of damage"

I have considered the cases cited and find that an award of Kshs.3,200,000/= is sufficient compensation to the plaintiff. I award the same.

Aggravated/Exemplary damages

The plaintiff also requested aggravated damages of Kshs.1,500,000/= This court, based on the damages claimed by the plaintiff, which demonstrate the aggravated circumstances of how the defendant's midwife caused her pain and anguish, considers the claim to reflect aggravated circumstances. The other claims are the painful experience that the plaintiff went through due to the wanton negligent acts of the defendant. I would therefore not hesitate to award her aggravated damages for that. Aggravated damages are meant to compensate the plaintiff for the additional injury going beyond that which flowed from the initial injury. The ruling by the committee indicated that a nurse administered oxytocin to the patient without the doctor's order. The committee deemed such a prescription to be inappropriate and contrary to expected standards of care. They also expressed the opinion that the administration of oxytocin infusion in a patient with obstructed labor precipitated uterine rupture, and the impacted fetal head may have impeded the proper flow of blood through the vaginal canal. The nurse had administered oxytocin which she should not have administered in the first place. This court finds an award of Kshs.1,000,000/= adequate to compensate the plaintiff.

Costs of the suit and interest

The court awards the plaintiff the costs of the suit and interest.

CONCLUSION

- 1) The court finds the defendant culpable for professional negligence, thereby finding them liable for the resultant loss and damages suffered by the plaintiff.
- 2) The plaintiff is awarded general damages of Kshs.3,200,000/=
- 3) The plaintiff is awarded aggravated damages of Kshs.1,000,000/=
- 4) The plaintiff is awarded the costs of the suit and interest.

HON C. K CHEPTOO

PRINCIPAL MAGISTRATE

JUDGEMENT is read and signed in the open court this **20TH** day of **FEBRUARY**
2024

PRESENT

Ms. Wanjiru Advocate present for the plaintiff

No Appearance for the defendants

Court Assistant Ms Joy

Court:

30 days stay of execution granted